Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION

Is a Psychology Degree Worth It in the AI Era?

The complete profile — the full scores by track, the six factors behind them, the honest downsides, and what to ask before you commit.

3.5

AI EXPOSURE · CLINICAL PSYCHOLOGIST (LICENSED)

where 10 is most at risk

Read this one first; send the student version to them.

HIGHLIGHTING IS COLOUR-CODED

THE FINDING EXPOSURE PROTECTION METHOD & HONESTY

Before you read this — a note for parents

You're likely reading this before your child is. That's normal, and it's worth pausing on how you use it.

Don't lead with the scores. If you open the conversation with a risk number, they either get frightened and shut down, or defensive about a decision they've already made.

The better approach: read this yourself first, then share it as *information you both get to react to*.

What this is. A facts document, not a recommendation. We've set out what's known, where the evidence is contested, and where we might be wrong.

A practical note: there's a separate short version written directly to them. Send them that one.

And one thing specific to psychology. This is the profile where the headline score is most misleading, in both directions. Licensed clinical practice is among the safest careers we measure. A psychology degree on its own is among the more exposed. The entire decision lives in the gap between those two, and that gap is about commitment to a long path rather than about AI.

If your child wants to be a therapist, this document is genuinely encouraging. If they're drawn to psychology as a subject, it's a different and more careful conversation.

The short answer

The licensed path is excellent. Most psychology graduates never reach it.

A licensed clinical psychologist scores **3.5** out of 10. A psychology degree with no further qualification scores **7.1**.

That 3.6-point spread is the largest gap in this index between destinations reached from a single degree. Nothing else we score varies this much depending on what a graduate does next.

And the number that should frame everything below: only about 14% of psychology bachelor's holders go on to graduate study in psychology.

The scores

DESTINATION	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Clinical psychologist (licensed)	2.8	3.2	**3.5**	+0.7	Low
Counselor / psychotherapist (licensed)	3.2	3.5	**3.9**	+0.7	Low
School psychologist	3.2	3.6	**4.0**	+0.8	Low–Moderate
Research / academic psychology	5.4	5.9	**6.5**	+1.1	Moderate–High
Organizational / HR-adjacent roles	5.9	6.5	**7.0**	+1.1	High
Degree only, no further qualification	6.1	6.6	**7.1**	+1.0	High

Scores run 1–10, *where* 10 *is most at risk*. For context: *the median profession sits around* 5.5, *bedside nursing scores* 2.8, *classroom teaching* 3.6, *and entry-level software development* 8.1.

Read the table this way: the top three rows all require a licence. The bottom three do not. The licence is doing essentially all of the protective work.

Why the licensed path scores low — the six factors

How much of this job can AI already do?	**6.8**
How hard will it be to land that first job?	**2.5**
Does it have to be done in person, with your hands?	**6.5**
Does someone need a human they can trust and hold responsible?	**9.5**
Does the law require a licensed human?	**9.5**
How often does the job hit genuinely new, high-stakes situations?	**9.0**

EXPOSURE RAISES THE SCORE　PROTECTION LOWERS IT

Here's how a licensed clinical psychologist answers them.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

A surprising amount of the surrounding work — and almost none of the therapy.

Session notes and documentation. Intake screening and scheduling. Progress tracking and outcome measures. Psychoeducational material. Standardized assessment scoring.

What does not automate: the therapeutic relationship, sitting with someone in genuine distress, judgment when a client is at risk, and legal accountability for a patient's safety.

We rate this 6.8 — higher than people expect. A great deal of what fills a clinician's week is automatable. It simply isn't the part that matters.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Once licensed, easy. Getting licensed is the filter.

Licensed clinical and counselling roles show unemployment as low as 1.3%, against roughly 2.5% for graduates generally. Demand comfortably exceeds supply and has for years.

But this factor measures the route in, and psychology's route is long: graduate study plus supervised practice hours, typically **six to eight years** in total. Much of that period is low-paid or unpaid, which is a financial filter rather than an academic one.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

Moderately — and less than most people assume.

Therapy is increasingly delivered remotely, and that works clinically. So psychology has less physical protection than nursing or medicine. It compensates elsewhere.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

This is the highest rating we give any career in this index, and section 6 explains why.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively — and, unusually, the law is currently moving *toward* the profession rather than away from it.

Several US states have moved to prohibit AI systems from delivering licensed therapy. This is one of very few places in this index where regulation is being written to protect a profession rather than quietly eroding around it.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Constantly. Every client is a different history, and risk judgments carry consequences that cannot be undone.

The trust factor — and the limit of what it protects

We give psychology **9.5** on human trust and accountability. That's the highest rating on any factor for any career in this index, and it deserves explaining — because understanding what it protects also reveals what it doesn't.

Two things sit inside this factor. *Trust*: does the work require a person present in a way another person can rely on? *Accountability*: must a human own the outcome when something goes wrong?

Therapy scores at the top of both, and for a reason that has no parallel elsewhere in this index. In most professions the relationship is a delivery mechanism for the service. In therapy the relationship is the treatment. A surgeon's patient wants competence. A therapy client needs to be known by someone. And when a client is at risk of harm, a licensed human carries duty-of-care obligations no system can hold.

But notice precisely what this protects.

It protects the *practice* — the requirement that a licensed human performs the work. It does not protect the *volume* of that work.

"Therapy and companionship" is now among the most common uses of general-purpose AI. A substantial share of young people report using chatbots for emotional support, mostly without telling anyone. None of that competes with a licensed therapist under the same rules — it isn't therapy, it isn't regulated, and it carries no accountability. But it may, over time, change how many people seek professional help at all.

Our framework measures whether a human must do the work. It cannot measure how many humans are needed.

That is the single clearest gap in our methodology, and psychology is where it shows. We'd rather name it than let a reassuring score do work it hasn't earned. A profession can be legally protected and still shrink.

What we're watching: whether referral volumes and waiting lists move in either direction over the next few editions. If demand for professional care falls while the licence holds, our score is measuring the wrong thing and we'll say so.

What the trend shows

Licensed clinical: 2.8 → 3.2 → 3.5. Up 0.7 points, and nearly all of it documentation.

Degree only: 6.1 → 6.6 → 7.1. Up 1.0.

The gap has widened from 3.3 points to 3.6. Both ends are drifting, and the unlicensed end faster — for the same reason business is drifting. Without a licence, a psychology graduate enters the general graduate market and carries whatever exposure that market carries.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Human trust / accountability	**Highest in the index**	**Durable**	The relationship is the treatment. Socially and legally rooted.
Regulatory / licensing	Very strong	**Most durable, and strengthening**	Several states are actively legislating to reserve therapy to licensed humans.
Judgment under novelty	Strong	**Eroding slowly**	Risk assessment and formulation are not automating meaningfully.
Physical / embodied	Moderate	**Weakest here**	Remote delivery already works, so there is less physical protection than in nursing or medicine.

The honest read. Psychology's protections are unusually durable — but they're all attached to the licence. A licensed clinician is about as protected as anyone in this index. An unlicensed graduate has none of it.

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing up session notes	Reviewing an ambient draft
Administering and scoring assessments	Interpreting results that arrive scored
Screening at intake	Triaging cases that arrive pre-screened
Tracking outcomes manually	Reading continuous outcome data
The therapy itself	Unchanged

The last row is the important one. In psychology the automatable layer and the therapeutic layer are cleanly separable in a way they aren't in most professions.

Two caveats. Verification becomes a clinical responsibility — an inaccurate record in mental health care is a safety issue. And clinicians increasingly need to be able to discuss what a client has been doing with a chatbot, which is a genuinely new competency.

Human strengths that will matter most

1. **Presence** — being reliably, attentively with another person. The core of the work and the least automatable thing in this index. 2. **Risk judgment** — recognising when someone is in danger. High-stakes, low-precedent, and the reason accountability attaches to a human. 3. **Accountability** — carrying duty of care. 4. **Formulation** — building an explanation of why *this* person is struggling in *this* way. Genuinely novel every time. 5. **Discussing AI use with clients** — new, unavoidable, and barely taught yet.

Other things to consider about this job

What's genuinely good about it

The licensed work is deeply valued by the people who do it. Practitioners describe it in terms of vocation rather than employment, and the reasons are consistent:

- **Depth of relationship** — few careers involve knowing people this well

- **Visible change over time** — the work produces results you can watch happen across months
- **Intellectual substance** — formulation is genuine problem-solving
- **Autonomy** — private practice is a realistic destination
- **Demand** — mental health need substantially exceeds provision in most systems

The practical case: low unemployment once licensed, flexible delivery including remote work, and a career that can be practised well into later life.

What's genuinely hard about it

The length and the money. Six to eight years to licensure, much of it low-paid or unpaid supervised practice. This is the real filter, and it is financial rather than academic. It excludes people who cannot afford several years of low income in their twenties.

The emotional load is sustained rather than episodic. This is work with people in genuine distress, repeatedly, over decades. Vicarious trauma is a recognised occupational risk.

Not everyone gets better, and sitting with that is harder than students expect.

The undergraduate degree does not qualify you for anything clinical. Worth stating plainly because many students don't realise it until year three.

Who this work suits — and who it doesn't

Psychology tends to suit people who:

- **Can be present without fixing.** The hardest and most important trait — much of therapy is tolerating someone else's distress without rushing to resolve it.
- **Are comfortable with slow progress**
- **Have genuine emotional stability**, since the work draws on it continuously
- **Are interested in people specifically**, rather than in the science of behaviour in the abstract
- **Can sustain a long, expensive path**

It's harder for people who:

- Need **quick, visible results**

- Are drawn to it primarily to **understand their own experience** — common, understandable, and not on its own a durable foundation for a career
- Cannot **afford the training years**, which is a real constraint deserving an honest conversation

What else is moving this market

Demand exceeds supply almost everywhere, and workforce projections generally show psychology demand outpacing the supply of qualified practitioners.

The training bottleneck is funded placements, not applicants. Supervised-hours capacity limits how many people qualify.

Regulation is moving in the profession's favour, which is rare in this index.

And the honest counterweight: psychology is the fifth most popular undergraduate major, with no cap on numbers. The undergraduate market is crowded; the licensed market is not. Those are two different labour markets sharing a name.

The number that should frame the decision

Everything in this profile reduces to one question: will your child complete the whole path?

Around 14% of psychology bachelor's degree holders go on to graduate study in psychology. A further 30% or so pursue graduate degrees in other fields. The remainder enter the general graduate market with a psychology degree.

That isn't a failure rate. Plenty of those graduates do well, and psychology is genuinely useful preparation for many things. But it means the modal outcome of a psychology degree is not clinical practice — it is a general graduate career informed by psychology.

Which reframes the decision honestly:

- **If they're committed to the licensed path**, psychology is an excellent choice and the score of 3.5 is the relevant one.
- **If they're uncertain about graduate study**, the honest comparison isn't "psychology versus other degrees." It's "the general graduate market versus other degrees" — and there, the relevant score is 7.1.

The question worth putting to any department: what proportion of your undergraduates reach licensed practice, and what are the rest doing five years out? The answer is usually smaller than families expect, and departments rarely volunteer it.

The AI-native advantage — what to actually do about it

The situation: clinical psychology is one of the few fields where AI arrives as *both* a workflow tool and a competing behaviour. Practitioners need to handle both.

WHAT AN AI-NATIVE PSYCHOLOGIST LOOKS LIKE

1. Verify. Catching when an ambient session note misrepresents what happened. In mental health an inaccurate record is a safety issue.

2. Discuss AI use clinically. Clients — especially younger ones — are already using chatbots for emotional support. A clinician who can ask about that without judgment, and work with what it surfaces, has a genuinely new and needed skill. Almost nobody is teaching it.

3. Understand the failure modes. Knowing how these systems behave with someone in crisis, where they validate inappropriately, and why that matters clinically.

4. Contribute to policy. Services are writing rules about AI in mental health care right now, and they need practitioner input.

5. Protect the relationship. Being able to articulate to a client, a service or a regulator why a human is required — psychology's deepest protection, defended in words.

CONCRETE PREPARATION

Before the degree: ask departments about progression to licensure — the answer varies enormously. Get any experience with people in distress: helplines, support work, volunteering.

During: treat research methods and statistics as core rather than as a hurdle — it's what separates a psychology graduate from someone who has read about psychology. Secure assistant or support-worker experience early; it's the bottleneck for graduate applications.

On the path: notice how services use these tools and where clinicians work around them.

The routes in

ROUTE	TYPICAL LENGTH	NOTES
Clinical psychology doctorate	3–6 years post-degree	Highly competitive; relevant experience usually required first.
Counselling / psychotherapy training	2–4 years post-degree	Often part-time and self-funded; a faster route to licensed practice.
School psychology	2–3 years post-degree	Persistent shortages; scores well at 4.0.
Psychiatric or mental health nursing	3–4 years total	Worth serious consideration — clinical mental health work, licensed, far shorter and cheaper, and scores 2.8.
Assistant / support roles	Immediate	The usual stepping stone, and the main bottleneck.

Worth knowing: for someone drawn to mental health work rather than to psychology as a discipline, mental health nursing reaches licensed clinical practice in a fraction of the time and cost. It is consistently under-considered by psychology applicants.

Where a psychology degree can take them later

Genuinely broad: clinical and counselling practice, school psychology, research and academia, organisational psychology, user research, health services, policy, HR and people roles, criminal justice, and teaching.

The pattern that runs through this index applies exactly: the further from licensed clinical practice, the higher the exposure. Organisational and HR-adjacent roles score 7.0 — good careers, ordinary exposure.

What to look for in a psychology program

1. Progression to licensure. What proportion of undergraduates reach clinical training? This is the number that matters and the one most rarely published.

2. Research methods depth. Statistics and experimental design, taught properly and compulsorily.

3. Practical experience built in. Placements, clinics, assistant work. Graduate applications need experience, and the degree should help produce it.

4. Clinical exposure and honest advising. Does the department tell students plainly how competitive doctoral training is?

5. Outcomes for the other 86%. Where do graduates who don't continue in psychology end up?

Questions to ask a psychology department

On progression:

- What proportion of your graduates enter clinical or counselling training within five years?
- What support exists for securing assistant psychologist roles?

On the curriculum:

- Is research methods compulsory, and how much of the degree is quantitative?
- What placement or clinic experience is built in?

On honesty:

- What are graduates who don't pursue psychology doing five years out?
- How competitive is doctoral training, and what does a successful applicant's profile look like?

On AI:

- How does the curriculum address AI in mental health — both as a clinical tool and as something clients are already using?

Red flags: no data on progression to licensure; research methods optional; no placement provision; vagueness about non-clinical outcomes.

Go deeper — further reading

- **The Labor Market for Recent College Graduates** — *Federal Reserve Bank of New York*. Look up psychology's unemployment and underemployment rates against nursing and engineering. Free, quarterly, and from a central bank rather than a careers site.
- **What Can You Do with a Bachelor's in Psychology?** — the source of the 14% progression figure, and a realistic account of where a psychology bachelor's actually leads.
- **Psychology and the Future: Two Decades of Graduates and Where They Work** — *North Carolina Department of Commerce*. Twenty years of tracked outcomes for psychology graduates from a state system — unusually good longitudinal data, and a state government rather than an industry body.

The counterargument — read this one:

> The strongest challenge to our licensed score isn't that AI will replace therapists. It's the opposite direction: that our 3.5 is **too optimistic**, because it measures the wrong thing.
>
> Our framework asks whether a human must legally do the work. In therapy the answer is yes and is being reinforced by legislation. But if a meaningful share of people who would previously have sought professional help instead turn to a free, always-available chatbot, then the profession stays protected while its client base thins. Regulation protects the practitioner from substitution *inside* the system; it does nothing about substitution *before* someone enters the system.
>
> **Where we land:** we think this is the most serious unresolved question in this index, and we've written it into section 6 rather than burying it here. We hold the score because our framework measures what it measures — and we state plainly that a profession can be legally protected and still shrink. We'll be watching referral volumes and waiting lists, and we'll report it if they move.

Bottom line

Psychology is two careers wearing one name.

The licensed path is genuinely excellent — 3.5, protected by the highest trust rating in this index, comprehensive licensure, and regulation that is currently strengthening rather than eroding. Low unemployment, real demand, work that people describe as vocation.

The degree on its own is an ordinary graduate credential — 7.1, and around 86% of psychology graduates don't continue in the field.

So the decision isn't about AI at all. It's about whether your child will complete a six-to-eight-year path, much of it low-paid. If yes, this is one of the better choices available. If uncertain, they should compare psychology against other general degrees rather than against the career it appears to promise.

And one option worth putting in front of anyone drawn to the clinical work rather than the discipline: mental health nursing reaches licensed practice in three to four years, scores 2.8, and is consistently overlooked by exactly the students who would be good at it.

Discussion questions

Part 1 — About the work itself

1. What made psychology appealing? Push past "I find people interesting" to something specific. 2. Do they want to *practise* — sit with people in distress, week after week — or do they find the subject fascinating? Both are valid; only one is a career path. 3. What does a Tuesday look like in the job they're imagining? 4. Can they be present with someone's distress without rushing to fix it? Think of a real example.

Part 2 — Reacting to the findings

5. Licensed clinical scores 3.5; the degree alone scores 7.1. Does that change anything? 6. Around 14% of psychology graduates continue in psychology. Was that surprising? 7. The profile says a profession can be legally protected and still shrink. What do they make of that?

Part 2b — The harder conversation

8. **Six to eight years, much of it low-paid.** Have you both worked through what that means financially — where they'd be living, on what income, at what age? 9. What's the plan if they don't get onto doctoral training? It's highly competitive, and many don't first time. 10. Have they looked honestly at whether the interest is in helping people or in understanding themselves? Both are common; only the first sustains a career.

Part 2c — The other side

11. Of what practitioners find rewarding — depth of relationship, visible change, formulation, autonomy — which two matter most? 12. Looking at "who this work suits": which items sound like them? Answer separately, then compare. 13. How do they cope with sustained emotional load rather than occasional stress?

Part 2d — The AI question, practically

14. Clients are already using chatbots for emotional support without telling anyone. How would they want to handle that as a clinician? 15. Does the substitution argument in section 6 worry them, or does it feel remote?

Part 3 — Testing it against reality

16. **The most valuable single action:** find a working therapist or clinical psychologist and ask what the path actually took, financially and personally, and whether they'd do it again. 17. Can they get real experience with people in distress before committing — helpline volunteering, support work, care work? It answers the fit question faster than anything else.

Part 4 — About the program

18. Ask each department the progression questions from section 13. Compare the willingness to answer. 19. Is research methods compulsory? How much of the degree is quantitative?

Part 5 — Widening the frame

20. If psychology weren't available, what would they choose? 21. **Have they looked at mental health nursing?** Licensed clinical mental health work, three to four years, scores 2.8. It's the most under-considered alternative in this whole index. 22. Do they want *psychology*, or what psychology is made of — helping people, understanding behaviour, research, or working with distress? Those lead to genuinely different places.

Part 6 — For the parent, alone

23. Am I comfortable supporting six to eight years of training, and have I said so honestly either way? 24. If they don't get onto clinical training, will I treat that as a failure? They'll read whatever I actually feel. 25. What's the one thing I most want them to understand — in a sentence, without a number in it?

This is analysis and informed judgment about a fast-moving situation. For psychology specifically, we've flagged that our framework measures whether a human must do the work, and cannot measure how many humans are needed — and that for this profession, the second question may matter more.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Psychology

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

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1. The formula

```
Risk = 0.35 × Automatability
     + 0.15 × EntryErosion
     + 0.15 × (10 − Physical)
     + 0.15 × (10 − Trust)
     + 0.10 × (10 − Regulatory)
     + 0.10 × (10 − Judgment)
```

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

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2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-seven careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human*?

RATING	ANCHOR
0–2	No licence exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent*, *where being wrong is costly*?

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

CLINICAL PSYCHOLOGIST (LICENSED) — 3.5 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.0	6.0	**6.7**	2.34
Entry-path erosion	15%	2.0	2.4	**2.8**	0.42
Physical / embodied	15%	6.5	6.5	**6.5**	0.53
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.5	9.5	**9.5**	0.05
Judgment under novelty	10%	9.0	9.0	**9.0**	0.1
				Total	**3.51 → 3.5**

COUNSELOR / PSYCHOTHERAPIST (LICENSED) — 3.9 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.7	6.4	**7.4**	2.59
Entry-path erosion	15%	2.2	2.6	**3.0**	0.45
Physical / embodied	15%	6.0	6.0	**6.0**	0.6
Human trust / accountability	15%	9.5	9.5	**9.5**	0.07
Regulatory / licensing	10%	9.2	9.2	**9.2**	0.08
Judgment under novelty	10%	8.8	8.8	**8.8**	0.12
				Total	**3.92 → 3.9**

SCHOOL PSYCHOLOGIST — 4.0 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.5	6.4	**7.4**	2.59
Entry-path erosion	15%	2.2	2.6	**3.0**	0.45
Physical / embodied	15%	6.0	6.0	**6.0**	0.6
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.0	9.0	**9.0**	0.1
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**4.0 → 4.0**

RESEARCH / ACADEMIC PSYCHOLOGY — 6.5 (MODERATE–HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.8	6.0	**7.4**	2.59
Entry-path erosion	15%	5.0	5.6	**6.2**	0.93
Physical / embodied	15%	2.0	2.0	**2.0**	1.2
Human trust / accountability	15%	5.5	5.5	**5.5**	0.67
Regulatory / licensing	10%	2.5	2.5	**2.5**	0.75
Judgment under novelty	10%	6.5	6.5	**6.5**	0.35
				Total	**6.49 → 6.5**

ORGANIZATIONAL / HR-ADJACENT ROLES — 7.0 (HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.0	6.4	**7.6**	2.66
Entry-path erosion	15%	5.5	6.2	**6.8**	1.02
Physical / embodied	15%	1.5	1.5	**1.5**	1.27
Human trust / accountability	15%	5.0	5.0	**5.0**	0.75
Regulatory / licensing	10%	1.5	1.5	**1.5**	0.85
Judgment under novelty	10%	5.5	5.5	**5.5**	0.45
				Total	**7.0 → 7.0**

DEGREE ONLY, NO FURTHER QUALIFICATION — 7.1 (HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	6.1	**7.3**	2.55
Entry-path erosion	15%	5.8	6.4	**7.0**	1.05
Physical / embodied	15%	1.5	1.5	**1.5**	1.27
Human trust / accountability	15%	4.5	4.5	**4.5**	0.82
Regulatory / licensing	10%	1.0	1.0	**1.0**	0.9
Judgment under novelty	10%	5.0	5.0	**5.0**	0.5
				Total	**7.1 → 7.1**

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Clinical psychologist (licensed)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	5.0	2.0	0.75	**2.8**
2025	6.0	2.4	0.75	**3.2**
Fall 2026	6.7	2.8	0.75	**3.5**

Movement decomposition: automatability contributed **+0.59** and entry-path erosion **+0.12**, for a total of **+0.7** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labour market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

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7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.